

ECR Post Implementation - Clinical Staff

Emergency Departments LEading the transformation of Alzheimer's and Dementia Care, or ED-LEAD for short, is a large-scale pragmatic randomized control clinical trial focused on improving emergency care and follow-up for persons living with dementia and their care partners through evidence-based clinical programs.

This is a questionnaire designed to be completed by clinical staff within the Emergency Department. The tool includes questions to assess ED-LEAD clinical decision support tools perceived usefulness and acceptance.

When completing the questionnaire, you can leave blank any questions that you do not want to answer. Remember that your responses are strictly confidential and will be closely guarded. Please try to answer all of the questions.

Please respond to the following statements based on your experience using the ED-LEAD alerts at your health system

Health system name

- ☐ Advocate Aurora Health
- ☐ Atrium Health Wake Forest
- ☐ Brigham & Women
- ☐ Henry Ford Health System
- ☐ Harvard Mass General
- ☐ MAYO Clinic
- ☐ Mount Sinai
- ☐ Northwell
- ☐ Northwestern
- ☐ Ochsner
- ☐ Pittsburgh
- ☐ Rochester
- ☐ Colorado

Please select the site name

- ☐ Austin
- ☐ Baptist-Ochsner
- ☐ Baptist-Atrium
- ☐ BayCare
- ☐ Brigham & Womens
- ☐ Central DuPage
- ☐ East
- ☐ Glen Cove
- ☐ Greeley
- ☐ High Point Regional
- ☐ Highland
- ☐ Huntington
- ☐ Jackson
- ☐ Lake Forest
- ☐ LJ Forest Hills
- ☐ LJ Hospital
- ☐ Lincoln
- ☐ Longs Peak
- ☐ McKeesport
- ☐ Mercy
- ☐ Morningside
- ☐ New Orleans
- ☐ North Shore
- ☐ Northwestern Memorial
- ☐ Noyes
- ☐ Palos
- ☐ Passavant
- ☐ Poudre Valley
- ☐ Queens
- ☐ Rochester
- ☐ Sheboygan County
- ☐ St Bernard Parish
- ☐ St Lukes South Shore
- ☐ Staten Island North
- ☐ Union
- ☐ University City
- ☐ West Bank
- ☐ West Bloomfield
- ☐ Wyandotte

Please indicate your agreement or disagreement with the following statements, considering your main organization:

	Strongly Agree	Agree	Neither Agree nor Disagree	Disagree	Strongly Disagree
1. Among my colleagues, I am usually one of the first to find out about a new diagnostic test, treatment or care process	☐	☐	☐	☐	☐
2. After we make changes to improvement, we evaluate their effectiveness	☐	☐	☐	☐	☐

3. Our procedures and systems are good at preventing errors from occurring

☐☐☐☐☐

4. We are innovative

☐☐☐☐☐

What do you think about the sufficiency of information you received about ED-LEAD implementation?

What do you think about the accuracy of the information you received about ED-LEAD implementation?

(Vague) 1

☐

2

☐

3

☐

4

☐

5

☐

6

☐

7 (Precise)

☐

What do you think about the usefulness of the information you received about ED-LEAD implementation?

Please rate how often you use the ED-LEAD components

	Never	Rarely (a few times a year or less)	Sometimes (once a month)	Often (once a week)	Very Often (multiple times per week)	Always (daily or almost daily)
How often do you use ED-LEAD introductory huddle alert?	☐	☐	☐	☐	☐	☐
How often do you use the ED-LEAD Dementia Navigator assessments?	☐	☐	☐	☐	☐	☐
How often do you identify and document care partner?	☐	☐	☐	☐	☐	☐
How often do you use ED-LEAD discharge huddle alert?	☐	☐	☐	☐	☐	☐

Why do you not use the ED-LEAD introductory huddle?
Please check all that apply.

- ☐ I was never informed about the ED-LEAD introductory huddle
- ☐ I do not think the information in the ED-LEAD introductory huddle is very useful for me
- ☐ The ED-LEAD introductory huddle is too complicated to use
- ☐ I do not have enough time to use the ED-LEAD introductory huddle
- ☐ Using the ED-LEAD introductory huddle does not fit into my workflow
- ☐ Other (please specify)

Please specify why you do not use the ED-LEAD introductory huddle.

Why do you not use the ED-LEAD Dementia Navigator assessments? Please check all that apply.

- ☐ I was never informed about the ED-LEAD Dementia Navigator and assessments
- ☐ I do not think the information in the ED-LEAD Dementia Navigator and assessments is very useful for me
- ☐ The ED-LEAD Dementia Navigator and assessments is too complicated to use
- ☐ I do not have enough time to use the ED-LEAD Dementia Navigator and assessments
- ☐ Using the ED-LEAD Dementia Navigator and assessments does not fit into my workflow
- ☐ Other (please specify)

Please specify why you did not use the ED-LEAD Dementia Navigator assessments.

Why do you not collect care partner information? Please check all that apply.

- ☐ I was never informed about collecting care partner information
- ☐ I do not think the information in the care partner information is very useful for me
- ☐ The care partner information documentation tool complicated to use
- ☐ I do not have enough time to collect and document care partner information
- ☐ Using the care partner alert does not fit into my workflow
- ☐ Other (please specify)

Please specify why you do not collect care partner information.

Why do you not use the ED-LEAD discharge huddle alert? Please check all that apply.

- ☐ I was never informed about the ED-LEAD discharge huddle alert
- ☐ I do not think the information in the ED-LEAD discharge huddle is very useful for me
- ☐ The ED-LEAD discharge huddle is too complicated to use
- ☐ I do not have enough time to use the ED-LEAD discharge huddle
- ☐ Using the ED-LEAD discharge huddle does not fit into my workflow
- ☐ Other (please specify)

Please specify why you do not use the ED-LEAD discharge huddle alert.

Training

Did you receive training for the use of ED-LEAD tools (Introductory huddle alert, Dementia Navigator, collection of Care Partner information, discharge huddle alert)?

- ☐ Yes
- ☐ No

	Poor	Fair	Good	Very Good	Excellent
Overall, how would you rate the training you received?	☐	☐	☐	☐	☐

The final set of questions are about the usefulness of ED-LEAD components

	Strongly Agree	Somewhat Agree	Neutral	Somewhat Disagree	Strongly Disagree
I rarely, if ever, use ED-LEAD components (huddle, navigator, collection of Care Partner information)	☐	☐	☐	☐	☐
ED-LEAD sometimes reminds me of things I otherwise would have forgotten	☐	☐	☐	☐	☐
ED-LEAD tends to slow down the Emergency Department workflow	☐	☐	☐	☐	☐
ED-LEAD has uncovered issues with patients that I might not otherwise have found out about	☐	☐	☐	☐	☐
I would rather not use the ED-LEAD alerts	☐	☐	☐	☐	☐
ED-LEAD has too many technical problems	☐	☐	☐	☐	☐
I often disagree with the recommendation ED-LEAD gives	☐	☐	☐	☐	☐
Reading and responding to ED-LEAD alerts takes too much time	☐	☐	☐	☐	☐
I repeatedly receive ED-LEAD alerts to which I have already responded	☐	☐	☐	☐	☐
Reading and responding to ED-LEAD alerts interferes with my workflow.	☐	☐	☐	☐	☐
ED-LEAD improves quality of care	☐	☐	☐	☐	☐
ED-LEAD alerts are useful in helping me care for my dementia related patients	☐	☐	☐	☐	☐
ED-LEAD alerts are relevant to the individual patients for which they appear	☐	☐	☐	☐	☐
ED-LEAD alerts I receive are clinically important	☐	☐	☐	☐	☐
ED-LEAD alerts help me improve the monitoring for and management for my dementia related patients	☐	☐	☐	☐	☐

ED-LEAD alerts help me reduce professional risk by preventing potential adverse events in my patients	☐	☐	☐	☐	☐
I find ED-LEAD alerts easy to understand	☐	☐	☐	☐	☐
The system makes it easy to respond to ED-LEAD alerts	☐	☐	☐	☐	☐

Demographic Information

Role	☐ Physician ☐ Resident ☐ Nurse ☐ Social Worker/Case manager ☐ Fellow ☐ ED-LEAD clinical champion ☐ Site PI ☐ APP (PA/NP) ☐ Other
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Please specify your role.

Gender	☐ Male ☐ Female ☐ Non-binary ☐ Prefer not to say ☐ Other (please specify)
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Please specify your gender.

Highest level of education you have completed?	☐ High school/GED ☐ Some college ☐ 2 year college degree (Associate) ☐ 4-year college degree (Bachelor, BA, BS, BSN) ☐ Master's degree ☐ Professional degree (MD) ☐ Doctoral Degree (PhD, DNP)
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How old are you?	☐ 34 or less ☐ 35-44 ☐ 45-54 ☐ 55+
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Are you Hispanic or Latino origin?	☐ Yes ☐ No ☐ Prefer not to answer
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What is your race?	☐ American Indian or Alaska Native ☐ Asian ☐ Black or African American ☐ Native Hawaiian or Other Pacific Islander ☐ White ☐ Other (please specify): ☐ Prefer not to answer
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What is your race?
